

Benchmark Experimental Proposal

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I have no known conflict of interest to disclose.

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Abstract

The question of what causes people to persist in problem pornography use when it results in dysregulation and emotional distress is the focus of this proposal. I hypothesize that morally incongruent pornography use is the output variable that effects the outcome variable of depression and that a new intervention of Emotionally Focused Individual therapy will reduce moral incongruence and depression. The study is also designed to determine how shame and God view mediate or moderate the outcome variable. A review of the literature validates a profitable study in this area.

Keywords: problem pornography, moral incongruence, depression, shame

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Pornography is a mode of fantasy and arousal that is used by many regardless of how it affects their relationships and individual emotions. There is no shortage of research designed to study pornography and why pleasure is so powerful. The research into the effects of pornography on relationships and individual emotional regulation leads many researchers to agree that pornography use can be harmful. The research into what keeps people returning to this harmful behavior is currently seeing conclusions relating to morally incongruent behavior more so than shame or addiction as concluded in the past (Grubbs et al., 2015). Finding treatments to intervene and give relief to those emotionally distressed is of concern to me as I write this proposal. In the proposed experiment below I will delineate the history of research, a design for addressing distress due to problem pornography use at the intersection of shame and moral incongruence, and state how this design will test the hypothesis that a treatment of Emotionally Focused Therapy for individuals will outperform a traditional treatment of Cognitive Behavioral Therapy with Relapse Prevention.

Literature Review

Problem pornography use has been of interest to researchers and clinicians since the turn of the century from what my research found. The early research pointed to a model of addiction to find reason for people to use pornography even when it caused distress (Carnes et al., 2004). In the decades since, researchers have been asking if hypersexual behavior and an addiction model are the best conclusions to draw (Grubbs et al., 2015; Reid et al., 2009, 2011; Volk et al., 2016). Overarchingly, the evidence is beginning to correlate moral incongruence with psychological distress/depression (Droubay et al., 2020; Floyd & Grubbs, 2022; Grubbs et al., 2010, 2015, 2019, 2022; Volk et al., 2016, 2019; Wright et al., 1989) when problem pornography

use is seen as morally wrong. There is also evidence that shame and one's view of God are variables interacting with moral incongruence and emotional distress to influence the degree of a person's dysregulation (Carnes et al., 2004; Floyd & Grubbs, 2022; Grubbs et al., 2015, Lawrence, 1997; Murray et al., 2007; Reid et al., 2009). The following experimental proposal hypothesizes that these variables will interact to increase depression in individuals using pornography when it is morally wrong for them.

Method

Participants and Procedures

Participants were 40 Christian, married, heterosexual men, over 40 who sought counseling for marriage distress at a major church counseling center. The couple's counselor recommended individual therapy in addition to couple's therapy to each man who was using pornography incongruently with his beliefs. Only if the men were reporting depression and incongruent pornography use, were they entered into the study.

Each participant was then randomly assigned to either a group participating in the clinic's empirically based treatment of a combination of Cognitive Behavioral Therapy and Relapse-Prevention Therapy (Kaplan & Krueger, 2010) or a new treatment of Emotionally Focused Therapy for individuals. Each participant was provided 8 sessions of treatments respectively. After 8 sessions of treatments, the PROMIS and God10 assessments were administered again.

Measures

Each participant was given pre-screening assessments of a short demographic survey, the CPUI, the TOSCA-3 to assess for shame (Lear et al., 2022), the Grubbs (2015) assessment for moral incongruence related to pornography use, and the PROMIS scale with Emotional Distress and depression subscales for depressive disorder (Pilkonis et al., 2014). The TOSCA-3

assessment is a 16-item self-report measure. The validity and reliability of the measure will indicate what degree of self-conscious shame a person is recognizing at the point of intake. Moral incongruence should be measured by at least two scales when the definition is engaging in behavior contrary to one's moral stance. One scale to measure the degree of belief and one scale to measure the degree of oppositional behavior. For moral incongruence related to pornography use, The Cyber Pornography Use Inventory (CPUI) could be used to measure participants' beliefs about pornography (Volk et al., 2019). The CPUI is a 40-item self-report with 6 subscale measures. It is a Likert-type report from strongly agree to strongly disagree (Grubs et al., 2010). The CPUI was designed to target online pornography use with allowances for religious moderation. Historically this measure has been proven in multiple studies (Grubs et al., 2010). The CPUI in multiple forms was shown by Floyd et al. (2022) to be a valid measure. This measure could be used in conjunction with Grubs et al. (2015) assessment designed specifically for a particular study to measure the amount of moral disapproval of pornography use. This measure asks four questions with answers on a Likert-type scale from 1 (not at all) to 7 (extremely). The study this measure was designed for yielded a Cronbach's alpha of above .70 in all but a few subscales. These two measures together would give a clearer understanding of the degree to which individuals are using pornography in a morally incongruent manner with beliefs or PPMI as it effects depression or negative affect. The PROMIS scale is included as part of the intake to measure pain, fatigue, emotional distress, sleep, physical functioning, and social participation (Pilkonis et al., 2014). PROMIS focuses on affect and cognitive aspects of depression over a seven-day period. The scale asks participants to self-report on questions using a five-point rating scale from 1 (never) to 5 (always) (Schalet et al., 2016). The Schalet et al. study mentioned above found the PROMIS scale to be a reliable source for measuring change in

negative affect after interventions were applied to participants. As each participant in the current study was assessed, the variables alluded to were tracked. Those variables and their interactions will now be discussed.

Variables

Moral Incongruity

One of the factors proposed to contribute to individuals seeking out pornography even when it is problematic and causes emotional dysregulation is morality (Duffy et al., 2016). Specifically, moral incongruence (Grubbs et al., 2019). Moral incongruence around pornography use is the leading factor in reporting dysregulation; more so than pornography use itself (Grubbs, et al., 2019). Using pornography for pleasure when it is believed by the user to be immoral, especially when one's religious views point to immorality, is the factor that increases shame and causes dysregulation according to the most recent research. Research like Grubbs et al. in 2019 discussed pornography-related problems arising from moral incongruence (PPMI) as broadly related to depression. One 2016 study with participants in convenience samples across a qualitative study resulted in participants experiencing greater shame over hypersexual behavior when their religious views called for this behavior to be a severely immoral act (Karaga et al.). While previous research focused on shame as a mediating factor and a generalizing view of how early attachment bonds caused addiction (Adams & Robinson, 2001), research in pornography use in the last five years has taken a turn toward individual use and how religion and morality effect perceived problems and addictions (Floyd & Grubbs, 2022). The cause for emotional dysregulation connected to pornography use is increasingly seen as moral incongruence. Shame (developed by early attachment styles), in these studies, becomes a moderating factor for emotional dysregulation including depression rather than the cause of emotional dysregulation.

Shame

Shame is an emotion related to the concept of the self. It can either be helpful in making decisions or harmful when linked to numerous psychological disorders. When shame is connected to problem pornography use it is often seen as harmful and leads to depression and other distressful emotions. Reid et al. (2009) found that individuals with hypersexuality disorder use withdrawal and negative self-criticism when attempting to counteract shame. These and other studies show there is evidence that shame and behaviors that cause dysregulated emotions are related. For example, shame was proven to be a positive predictor of hypersexual behavior in a study of pornography users already in treatment for hypersexuality (Gilliland et al., 2011). However, the most recent evidence points to shame as an outcome of problem behavior or as a mediator of the outcome of problem behavior rather than as the cause of problem behavior (F. Volk, personal communication, October 13, 2022). As shame is considered part of the dysregulation of emotions in connection to problem pornography use, one must account for the degree to which shame is precipitating or mediating distressful emotions along with one's image of God.

God Image

God image can be divided into two basic categories: I see God as cruel, or I see God as close. Whether an individual sees God as cruel or close is determined less by psychological constructs and more by experience with representations of God in one's life (Lawrence, 1997). For example, one has experiences with one's biological father that related back to the concept of who God is as a person (Lawrence). An emotional wound or broken bond perceived to be caused by a biological father could easily be projected onto God as a Father wound. In the study referenced above by Lawrence, the prediction is made that individuals with low self-esteem

would have a negative or wounded view of God. This is in keeping with the current understanding of how problem pornography causes emotional dysregulation. That is, shame and religion do not cause emotional dysregulation, but rather shame and one's view of God mediate and moderate emotional dysregulation when problem pornography use is incongruent with one's moral views causing emotional distress or depression.

Depression

For the purposes of this paper, clinically significant distress, distress that leads individuals to seek help, in connection to problem pornography use was the focus of the literature search. With this criterion in mind, Droubay et al. (2020) found evidence that, when pornography use is viewed as morally wrong, shame is highly correlated to general distress. Their study showed that moral incongruence, shame proneness, and emotional dysregulation were all correlated with subjective distress. Past studies, such as Wright et al. (1989) yielded results that indicate shame, when moderated by self-image, causes depression. Additionally, where many studies have focused on depression and/or distress and their connection to internet sex, Volk et al. (2019) point out that self-reports of life satisfaction are often influenced by depressive symptoms. Both above studies connect a negative view of self (shame) with distress over pornography use. This leads to the hypothesis of this study that moral incongruence is correlated with psychological distress/depression (Droubay et al., 2020; Floyd & Grubbs, 2022; Grubbs et al., 2010, 2015, 2019, 2022; Volk et al., 2016, 2019; Wright et al., 1989) when individuals view pornography use as morally wrong or incongruent with held beliefs. To measure the degree to which the above variables interact, I turn now to the best practices for statistical procedures.

Statistical Procedures

As the hypothesis in this proposed study states that as the output variable increases the outcome variable also increases it would be appropriate to use analysis of covariance (ANCOVA) to ensure internal validity. As this study has a single variable with two levels the control group serves as the dependent variable, and it is important to control for the differences between pretest scores of the dependent and independent variables (Jamieson, 2004). According to the Jamieson study above, when groups are assigned at random, as is the case with this study, ANCOVA is an appropriate and effective way to measure changes from pretest to posttest between the two groups while controlling for internal validity. With the study proposed being a between-group experiment design randomly sampled population, and I propose a pretest and posttest, ANCOVA is the most accepted way of testing for internal validity or the degree to which I can make further statements about the reality that one variable causes another variable to increase or decrease (Heppner et al., 2016).

Validity

Internal validity is an important factor in any study. If there is no relationship between the variables, then the study cannot prove nor disprove the hypothesis. As far as this study is concerned, the threats to internal validity have been limited by choosing 8 sessions to be a balance between seeing change and controlling for maturation. Measuring only twice, once in a pretest and once in a posttest, will control for ambiguous temporal precedence and testing bias. Controlling for history becomes somewhat instant in nature. For example, if a participant were to divorce during the process, I would need to complete the treatment but not include his test results in the analysis. Internal validity, therefore, seems to be an ongoing concern for this or any experiment.

Internal validity is not the only threat to the hypothesis of this design. There is no perfect design, however, checking this proposal for statistical validity appears to clear it from major threats. The sample size is adequate to measure change. The homogeneity is sufficient between sex, age, marital status, religious beliefs, and presenting problems to control for different levels of distress (Heppner et al., 2016). In addition to these factors, the measures chosen are empirically tested and proven to measure as proposed. Which is also a concern for construct validity.

The construct validity of this proposal can be controlled by utilizing both a double-blind method and ensuring that procedures for testing and treatments are followed closely. Bias in treatment resulting in differing treatments for participants could render the test results as flawed. Ensuring the treatment is proper and effective will also guide participants to complete treatment and not look elsewhere for treatment that could interact unknowingly with the results of the study. Construct and overall validity start with the population and sample chosen.

When controlling for external validity, my study utilizes random sampling from a homogenized volunteer population. This will allow me to generalize the study to a specific population but not a larger population. One critique of this experiment could be that differing genders, relationship statuses, or ages could respond differently to the treatment. However, for the most part, external validity has been accounted for by treatment consistency, setting consistency, and consistent measures. The experiment is not perfect but should yield measurable results for the testing of the hypothesis.

Conclusion

The proposed experiment hypothesizes that a new treatment targeting moral inconsistency and shame will result in improved outcomes over an existing treatment for men struggling with morally inconsistent problem pornography. The hypothesis is empirically appropriate considering the history of research and data supporting the proposed conclusions. The design of the study is homogenized sufficiently to control for validity and outcome measures. Considering the proven effectiveness of both proposed treatments, the design is ethical and can be assumed to be measurable. It is reasonable to conclude that a treatment designed to interact with shame and attachment bonds will be more effective when treating a disorder that is more closely related to affect than it is to cognition. I am prepared to offer further study in this area considering the small population and treatment recommendations following the statistical analysis. It is foreshadowed that suggestions for improvement upon this study will be gathered throughout the experiment.

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